

TRI-COUNTY PUBLIC OUTREACH COMMITTEE

Tri-County Service Center

123 NE 3rd Avenue, Suite 440
Portland, Oregon 97232-2901
Telephone: (503) 872-6731

Meeting Minutes November 28, 2001

Meeting Location: Tri-County Service Center
123 NE 3rd Avenue, Suite 440
Portland, Oregon 97232

Time: 9:00 a.m.

Members Present: Kevin Brice (Chair), Washington County
Sue Blatner, Industry – SuperBath Showroom
Guy Sperb, City of Oregon City
Shelley Ashenfelter, Home Builders Association
Ann Kohler, City of Portland

Members Absent: Ron Murray, Local 290/Tri-County Board

Guests: Anne Madden, Washington County
Jim Muir, Washington County
Jennifer Fouhy, Washington County

Staff: Joanie Stevens-Schwenger, Tri-County Service Center
Laurie A. McCluskey, Tri-County Service Center

Chair Kevin Brice called the meeting to order at 9:05 a.m.

1. APPROVE NOVEMBER 7 MINUTES

The committee reviewed the minutes.

Motion to approve the minutes. Motion approved

2. CONTINUE THE CREATIVE BRIEF PROCESS

See attachment A. The creative brief is a tool that will enable the group to construct an effective communications plan. The group worked on items 4 (c) through 11. Joanie and Ann briefly discussed some ideas for 12, 13 and 14. The group ran out of time and put these items on the agenda for the next meeting.

3. AGENDA FOR NEXT MEETING

- a. Complete the *Creative Brief* exercise.
- b. Develop a strategy to deliver the message to the target audiences.

The meeting adjourned at 11:05 a.m.

Next Meeting: **Wednesday, November 28, 2001**
9:00 p.m. – 11:00 a.m.

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Attachments:

- A.** Public Outreach Communications Project - *Creative Brief* – “Draft Work-in-Process” – November 7, 2001
- B.** Excerpts of Comments – November 7, 2001 Session

AMENDED IN ASSEMBLY AUGUST 16, 2010

AMENDED IN ASSEMBLY AUGUST 2, 2010

AMENDED IN ASSEMBLY JUNE 24, 2010

AMENDED IN SENATE MARCH 23, 2010

SENATE BILL

No. 1450

Introduced by Senator Simitian

February 19, 2010

An act to add Section 85213 to the Water Code, relating to water, *and declaring the urgency thereof, to take effect immediately.*

LEGISLATIVE COUNSEL'S DIGEST

SB 1450, as amended, Simitian. Water: Delta Stewardship Council: contracts.

The Sacramento-San Joaquin Delta Reform Act of 2009 establishes the Delta Stewardship Council, which is required to develop, adopt, and commence implementation of a comprehensive management plan for the Sacramento-San Joaquin Delta (Delta Plan) by January 1, 2012. The act provides that the council is the successor to the California Bay-Delta Authority and requires the council to administer contracts, grants, easements, and agreements made or entered into by the authority. The act prescribes the powers of the council, which include, among others, the power to enter into contracts, to hire employees, and to adopt regulations or guidelines.

Existing law prescribes requirements for selection of firms by state and local agency heads for architectural and engineering services and other professional services.

This bill would expressly authorize the council to contract for consultant services and architectural and engineering services to assist

with preparation of the Delta Plan and performance of certain responsibilities of the council. The bill would exempt ~~these contracts for consultant services~~ from specified provisions of law governing public contracting, ~~but would require the contracts to be entered into on a competitive bid basis, as specified.~~ *The bill would require contracts for architectural and engineering services to be entered into pursuant to the requirements for selection of firms by state and local agency heads discussed above, but would exempt those contracts from a requirement to adopt regulations prior to contracting.* The bill would require the council to establish procedures for contracting for consultant services and architectural and engineering services.

This bill would declare that it is to take effect immediately as an urgency statute.

Vote: ~~majority~~^{2/3}. Appropriation: no. Fiscal committee: yes. State-mandated local program: no.

The people of the State of California do enact as follows:

1 SECTION 1. Section 85213 is added to the Water Code, to
 2 read:
 3 85213. (a) The council may contract for consultant services
 4 and architectural and engineering services to assist the council
 5 with all of the following functions:
 6 (1) Preparation of the Delta Plan and associated environmental
 7 review.
 8 (2) Performance of the council's responsibilities, or the
 9 responsibilities of the Delta Independent Science Board, with
 10 respect to the Bay Delta Conservation Plan.
 11 (3) Determination of the consistency of state and local public
 12 agency actions with the Delta Plan.
 13 (4) Performance of scientific review to inform water and
 14 environmental decisionmaking in the Delta.
 15 (b) (1) Contracts entered into pursuant to subdivision (a) *for*
 16 *consultant services* shall be exempt from ~~Chapter 10 (commencing~~
 17 ~~with Section 4525) of Division 5 of Title 1 of the Government~~
 18 ~~Code, Article 4 (commencing with Section 19130) of Chapter 5~~
 19 ~~of Part 2 of Division 5 of Title 2 of the Government Code, Part 2~~
 20 ~~(commencing with Section 10100) of Division 2 of the Public~~
 21 ~~Contract Code, and any rules or regulations adopted pursuant to~~
 22 ~~those laws.~~ *Contracts entered into pursuant to subdivision (a) for*

1 *architectural and engineering services shall be entered into*
2 *pursuant to Chapter 10 (commencing with Section 4525) of*
3 *Division 5 of Title 1 of the Government Code, but are exempt from*
4 *the requirement in Section 4526 of the Government Code to adopt*
5 *regulations prior to contracting.*

6 ~~(2) Contracts entered into pursuant to subdivision (a) shall be~~
7 ~~entered into on a competitive bid basis.~~ The council shall not enter
8 into a contract pursuant to subdivision (a) *for consultant services*
9 unless at least three bids or proposals to do the work have been
10 evaluated by a competitive process.

11 (3) The council shall establish procedures for contracting for
12 services pursuant to subdivision (a), consistent with paragraphs
13 (1) and (2). The procedures shall be exempt from Chapter 3.5
14 (commencing with Section 11340) of Part 1 of Division 3 of Title
15 2 of the Government Code.

16 *SEC. 2. This act is an urgency statute necessary for the*
17 *immediate preservation of the public peace, health, or safety within*
18 *the meaning of Article IV of the Constitution and shall go into*
19 *immediate effect. The facts constituting the necessity are:*

20 *In order to ensure proper contracting by the Delta Stewardship*
21 *Council, it is necessary that this act take effect immediately.*

Fermi Summer School 2012 Application Form
(Submit via e-mail to fermischool@bigbang.gsfc.nasa.gov)

Applicant Name: _____

Applicant E-mail: _____

Institution: _____

Date or Anticipated Date of PhD: _____

Room preference

Double (\$1000): _____ **or Single (\$1500):** _____ (singles subject to limited availability)

Adviser Name: _____

Adviser E-mail: _____

If you would like to give a brief oral presentation of your work, please provide a title and abstract.

Title:

Abstract:

Statement of Interests

Please provide a brief summary of your current research activities and science interests
(Please do not exceed one page)

CME: Continuing Medical Education or Commercial Marketing Efforts?

Testimony submitted to the Senate Committee on Aging

Adriane Fugh-Berman MD
Director, PharmedOut
Department of Physiology and Biophysics
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Disclosure: Dr. Fugh-Berman has been a paid expert witness on behalf of plaintiffs in litigation regarding pharmaceutical marketing practices.

Continuing medical education (CME) is the pharmaceutical industry's most important marketing tool. The development of marketing messages for a drug starts seven to ten years before a drug is submitted for FDA approval. Many of the marketing messages that are developed for each product do not mention the drug at all. A 'pre-launch' marketing message might emphasize the importance of a specific physiologic process in order to set the stage for acceptance of a drug that affects that mechanism, or might create an unnecessary diagnostic distinction in order to establish a niche for a drug entering a crowded market.

Selling disease

CME is used to sell drugs by selling diseases. As a marketing article called "Proving the case for investing in CME," states: "*The most significant benefits for industry may include creating disease-state awareness and disease state significance.*"¹ Another marketing article notes that: "*CME activities are most valuable in introducing products early in their life cycles or for promoting mature brands with new indications and new clinical data.*"²

Industry-funded CME often emphasizes the severity or prevalence of specific conditions in order to prepare, or expand, a market.³ Manipulating physicians' understanding of the prevalence or severity of medical conditions can lead to overtreatment, and expose patients to the adverse effects of drugs without significant benefit.

The marketing messages imbedded in CME lectures, articles, on-line modules, and tests are never advertisements for a specific therapy because physicians will reject speakers or articles that obviously favor a specific drug. One marketing message might emphasize the risks of competing therapies; another the lack of evidence regarding an over-the-counter remedy.

CME is often used to promote unproven uses of a drug. While it is illegal for a drug company to promote off-label use of a drug, CME is not considered promotion and is not regulated by the FDA. Physicians can say whatever they want, so are used as mouthpieces for marketing messages that would be illegal coming from a company rep.

The academic physicians involved in industry-funded CME may protest, quite honestly, that they are expressing their independent opinions. However, they are chosen because what they are saying aligns with a product's marketing messages, and are supported only as long as they do so.⁴ A physician who expresses doubts about a

¹ Raichle L. [Proving the case for investing in CME](#). *Medical Marketing & Media*. Jun 1998; 33(6): 84.

² Bottiglieri D. [Crowd Pleaser](#). *Pharmaceutical Executive*. Sep 2001; 21(9): 122.

³ Fugh-Berman A, Melnick D. Off-Label Promotion, On-Target Sales. **PLoS Medicine** 2008;5 (10): e210 <http://medicine.plosjournals.org/perlserv/?request=get-document&doi=10.1371/journal.pmed.0050210>

⁴ Fugh-Berman A. Key opinion leaders: Thus are our medical meetings monitored. **BMJ** 2008;337:a789 http://www.bmj.com/cgi/content/full/337/jul15_1/a789

product's efficacy, concerns about its risks, or enthusiasm for a competing therapy will be unceremoniously dropped from a company's speaker's bureau.

Vetting bias in CME modules

There is little incentive for medical education and communication (MEC) companies or academic medical centers to attempt to ferret out bias in CME modules, because both depend on industry for survival. In 2007, about half of the \$2.5 billion spent on CME came from pharmaceutical manufacturers. Seventy-two percent of the income from events sponsored by medical education companies is from firms that manufacture FDA-regulated products. Academic medical centers are only a little better; 57% of CME income to medical schools comes from industry.

Although academic CME providers do their best to vet industry-funded modules for bias, such review focuses on the detection of obvious advertising, the use of brand rather than generic names, of drugs, and adequate disclosure of conflict of interest. No matter how vigilant they are, CME providers are not trained in recognizing marketing messages and cannot adequately assess industry-funded materials for bias.

As previously noted, marketing begins years before a drug reaches the market. Adequate vetting of CME modules would require knowledge of every drug, biologic, device, and diagnostic test that every company is marketing, developing, or considering. The task becomes even more daunting when mergers, acquisitions, co-marketing agreements and collaborative marketing arrangements for example, among companies selling drugs in the same class) are taken into account. Similarly, disclosures of financial conflicts of interest are impossible to interpret without knowing every company's marketing plan for every drug on the market or in the pipeline.

'Unrestricted' grants

Unrestricted educational grants to academic medical centers or MECs are no solution. Although the new PhRMA Code of Ethics does not allow sponsors (companies that fund the event) to suggest speakers, sponsors may indicate which topics they are interested in funding. "Unrestricted" grants provided for grand rounds and lunch conferences depend on a sense of obligation rather than a quid pro quo. When lists of recommended speakers are supplied to organizers, it is unstated, but nonetheless understood, that company-paid speakers will be included in the lecture series.

The organizing board of the CME program may include speakers selected and paid by the sponsor. The desired messages about particular products or diseases may be made clear. Organizers understand that sponsors or exhibitors may withdraw if they don't like the content. Not every speaker in a lecture series or conference is chosen by industry. Pharmaceutical companies refer to presentations with marketing messages as "message talks," and sponsored CME programs usually include talks that are not connected with the sponsor's product. Although these camouflage talks may involve independent speakers, organizers know that they must avoid inviting speakers who might criticize a sponsor's products or oppose a sponsor's marketing messages³.

Physicians will never hear about how a targeted disease is overdiagnosed or overtreated at a sponsored event. And rarely will they hear positive recommendations

for a competitor's product, a generic product, or a non-pharmacologic therapy at a sponsored program.

ACCME

The ACCME Standards for Commercial Support⁵ to discourage potential conflicts of interest are both weak and ignored. A 2007 annual report by the ACCME, however, showed that 29% of providers failed to comply with Standard 2, which requires that any individuals that control content of a CME activity disclose and resolve conflicts of interest.⁶ 27% of the providers did not comply with Standard 3, which includes guidelines to prevent those with commercial interest from dictating the content of a CME activity. And 36% of providers were noncompliant with Standard 6, which requires that all commercial support and financial relationships of authors be disclosed to CME participants before the activity.⁶

The ACCME does not screen CME activities, and does not encourage audience members to report commercial bias. Although written complaints are accepted, the process is burdensome. The ACCME would not tell us whether or not any providers have lost accreditation status due to commercial bias. Losing accreditation for any reason is rare; according to the annual reports published by the ACCME only one provider lost accreditation in 2005.

Conclusion

If sponsoring CME events did not increase product sales, drug companies would not do it. The large amount of commercial support poured into CME is in itself testimony that industry believes supporting CME is cost-effective. Industry influence on medical discourse limits the discussion to the most profitable therapies, which may not be best for patients. Industry-funded medical education is a contradiction in terms.

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Acknowledgments: Alicia Bell MS, Margaret Infeld, Clare Murphy, Stephanie Waterhouse, and Nicole Woodard provided background research.

About PharmedOut

[PharmedOut](#), a project based at Georgetown University Medical Center, uses academic research and drug industry insider perspectives to educate prescribers about how covert marketing techniques affect prescribing behavior. PharmedOut was launched through a grant from the [Attorney General Consumer and Prescriber Grant Program](#).

Our latest free, web-based, 3-CME credit educational module is called [The Pharmalyzer: Are you prescribing under the influence?](#) Our unique approach draws on academic and marketing materials

⁵ ACCME. Essential Areas and Elements; 2008 Available via the Internet: http://www.accme.org/dir_docs/doc_upload/f4ee5075-9574-4231-8876-5e21723c0c82_uploaddocument.pdf. Accessed June 30, 2009

⁶ ACCME [Standards for Commercial Support; 2008 Available via the Internet: http://www.accme.org/dir_docs/doc_upload/68b2902a-fb73-44d1-8725-80a1504e520c_uploaddocument.pdf. Accessed June 30, 2009.

combined with original interviews with industry insiders to create novel articles, videos, and educational materials. Our goal is to foster evidence-based, cost-effective prescribing and to decrease the adverse public health effects of inappropriate pharmaceutical promotion.

We provide original web-based [CME modules](#) and [links](#) to more than 150 free, non-industry-funded, web-based continuing medical education (CME) courses – enough for any physician, nurse, or pharmacist to complete annual continuing education requirements. Educational resources include our [slideshows](#), [Drug Ad Bingo](#) (a teaching exercise), [Fast Facts on Generic Drugs](#) (a factsheet for patients), and seven original videos, featuring industry insiders. Our [publications](#) include [Off-Label Promotion](#), [On-Target Sales](#), [Following the Script: How Drug Reps Make Friends and Influence Doctors](#), [Do New Drugs Increase Life Expectancy?](#), [Ethical Considerations of Publication Planning in the Pharmaceutical Industry](#), [Prescription Tracking and Public Health](#), [Key Opinion Leaders: Thus Are Our Medical Meetings Managed](#), and [Smoke and Mirrors](#).

ⁱ Raichle L. Proving [the](#) case [for](#) investing [in](#) CME. *Medical Marketing & Media*. Jun 1998; 33(6): 84.



TO: Executive Officers

FROM: NIH Day Care Board

SUBJECT: Appropriated Funds to Subsidize Child Care Costs

The NIH Day Care Board was established in 1989 with the mission of promoting quality child care services for NIH employees and advising the NIH with regard to child care issues and policies. To this end, we are asking the Executive Officers to support the implementation of recent legislation that permits agencies in the Executive branch to use appropriated funds to subsidize child care costs for their lower-income Federal employees. The intent is to make child care more affordable for these employees, for whom child care can consume up to 50 percent of their family's total income.

The NIH Day Care Board believes that the NIH would experience numerous significant benefits resulting from the implementation of a child care subsidy program. These include increased employee retention and productivity, reduced tardiness and absenteeism, and increased commitment and loyalty to the NIH. The following data illustrate these benefits:

- The Maryland Child Care Resource Network reports that workers miss more than seven workdays each year due to dependent care problems, (looking for care, unanticipated closings, transportation issues, etc.), and that when employers provide assistance with dependent care issues, absenteeism is reduced by 20 to 30 percent (Implementing Work/Life Programs Employer Tool Kit, Maryland Committee for Children/Maryland Child Care Resource Network, 1999).
- According to the 1997 National Study of the Changing Workforce, workers who perceive that the organizational culture is supportive of their family and personal needs are more committed to and more likely to remain with their employer (The 1997 National Study of the Changing Workforce, Bond, Galinsky, Swanberg, Families and Work Institute, 1998).
- In an assessment of the NationsBank child care subsidy program, the bank acknowledged that there was one-third lower turnover in staff and an increase in worker productivity because parents had fewer worries about child care (Implementing Work/Life Programs Employer Tool Kit, Maryland Committee for Children/Maryland Child Care Resource Network, 1999).

To date, six Federal agencies have implemented a child care subsidy program, and an additional six have submitted letters of intent to OPM. Within HHS, FDA, PSC, SAMSHA, and OS/AOA have all committed money for this purpose. All agencies which implemented the program in fiscal year 2000 will continue to provide it in fiscal year 2001.

Many will be expanding their programs because actual operating costs were *lower* than anticipated. In addition, many agencies have chosen to set a "not to exceed" spending cap to limit the impact on their agency budgets.

We encourage you to support the implementation of a child care subsidy program at the NIH. It is clear that NIH employees would benefit from such a program. Last year, the NIH Work and Family Life Center Child Care Resource & Referral service was contacted by 438 NIH employees. Fifty percent of NIH employees using their service reported added stress because of dependent care issues. An additional 40 percent of service users indicated that they had been absent from work in order to address dependent care issues. We need to show our support for our working parents who struggle with the enormous financial and emotional burdens of child care. Ultimately, our support will benefit the NIH by enhancing recruitment efforts, increasing job satisfaction, and improving retention and productivity in our current workforce. The quality of research and science administration that we do at the NIH is a direct result of the quality of the workforce and the quality of our lives, both at home and at work.

Should you have any questions or concerns, please direct them to Dr. Deborah Henken at (301) 435-6885 or Ms. Corliss Taylor at (301) 435-2651.

Sincerely,

/s/
Deborah Henken, Ph.D.
Co-Chair, NIH Day Care Board

/s/
Corliss A. Taylor, MPA
Co-Chair, NIH Day Care Board

Distribution:
Executive Officers, NIH

cc:
Acting Director, NIH
Deputy Director for Management, NIH
Director, OHRM
Director, ORS
NIH Day Care Board

This letter was signed and distributed to the NIH Executive Officers on January 15, 2001